Serplulimab, carboplatin and etoposide

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction: carboplatin Day 1 and etoposide Days 1–3

Active medicines: serplulimab + carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk and patient factors.
Dexamethasone antiemetic reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	National Cancer Control Programme (NCCP) NCIS V8 high-risk dedicated carboplatin AUC≥4 row One reconciled prescription for the multi-day course. Avoid duplicating corticosteroids when the day-specific etoposide support is reviewed.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS table does not specify an end day here; prescribe duration and suitability. Not a routine olanzapine-containing row.
Days 2–3 etoposide and Days 4–5 review	No automatically added duplicate course; confirm individual day-specific prescription	Before each etoposide day and for two days after the final chemotherapy day NCCP NCISV8 multi-day rule applies. Etoposide is low risk; the generic low-risk reference is ondansetron8mg PO daily with subsequent metoclopramide10mg PO TDS as needed, but it must be reconciled with the existing carboplatin course, QT risk and local policy rather than simply stacked. Day5 coverage must be explicitly reviewed.

Additional phase-specific support

- Use the dedicated carboplatin AUC≥4 high-risk reference (no routine olanzapine): aprepitant125mg Day1 then80mg Days2–3, ondansetron16mg Day1, dexamethasone12mg Day1 then8mg Days2–4; subsequent metoclopramide10mg TDS with duration/suitability prescribed.
- Etoposide is given on Days1–3. Reconcile support for each active treatment day and for two days afterwards; a fixed Day1 script alone is not a complete multi-day prescription.
- No routine infusion premedication is usually required unless there has been a previous hypersensitivity reaction. Have anaphylaxis facilities available.
- If oral etoposide is chosen, confirm capsule strengths, empty-stomach dosing, adherence and handling; no replacement dose after vomiting/missed dose without advice.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00908 v1b · p7–8 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-26 · SACTCheck v0.84.23 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Maintenance: serplulimab alone

Active medicines: serplulimab

Emetogenic risk: minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Serplulimab alone has minimal emetogenic potential: no routine continuation of the induction chemotherapy antiemetic course.
- Provide symptom-directed rescue if appropriate; investigate new nausea/vomiting for endocrine, immune and other causes.
- Continue cycle bloods, thyroid tests every3–6weeks and immune symptom vigilance; keep the alert card.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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